Supplementary Table 3. Assessment of study quality using MINORS tool [9]

Study	Clearly stated aim	Inclusion of consecutive patients	Prospective collection of data	Endpoints appropriate to the aim of the study	Unbiased assessment of the study endpoint	Follow-up period appropriate*	Loss to follow-up<5%	Prospective calculation of study size	Total score
Sharma and Maharshi, 2008 [15]	2	2	0	2	0	2	2	0	10
Jazrawi et al., 2011 [16]	2	0	0	2	0	1	2	0	7
Makin et al., 2012 [17]	2	0	0	2	0	2	2	0	8
Ramesh et al., 2013 [20]	2	0	0	2	0	2	2	0	8
Agarwal et al., 2013 [18]	2	0	0	2	0	0	2	0	6
Bai, 2013 [19]	2	0	0	2	0	2	2	0	8
Bang and Varadarajulu, 2016 [21]	2	0	0	2	0	2	2	0	8
Nabi et al., 2017 [22]	2	0	0	2	0	2	2	0	8
Nabi et al., 2019 [23]	2	0	0	2	0	2	2	0	9
Farr et al., 2020 [24]	2	0	0	2	0	2	2	0	8
Lal et al., 2020 [25]	2	0	0	2	0	0	2	0	6
Poddar et al., 2021 [27]	2	0	0	2	0	2	2	0	8
Seol et al., 2021 [28]	2	0	0	2	0	0	2	0	6
Ghoneem et al., 2021 [26]	2	0	0	2	0	0	2	0	6
Average score	2	0.1	0	2	0	1.4	2	0	7.6

The items are scored as 0 (not reported), 1 (reported but inadequate), or 2 (reported and adequate). The global ideal scores were 16 for non-comparative studies and 24 for comparative studies.
MINORS: methodological index for nonrandomized studies.
*≥1 year is considered adequate follow-up.